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The Skincare Investigation · 02

The Skin Skincare Forgot to Study

The beauty industry sells skincare to everyone. But the research behind it has not always studied everyone equally.

BOYOWA OLUGBO · MANAGING DIRECTOR, PRAXIS BRANDS

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Where beauty grows.

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BOYOWA

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Editorial packaging: the series label, subtitle, the headings "What Doctors Were Taught" and "The Missing Faces", and the final "Next" bridge are editorial additions. The body passages are drawn from the original research

A Note on Race, Skin, and What Follows

Everything from here concerns biology, and biology is where this subject goes wrong most easily. Usually in the direction of confirming whatever the reader already suspected.

So, plainly. Race is not a biological category. Human beings do not sort into biological races, and the overwhelming majority of traits show no consistent differentiation between the populations that racial labels describe.

Skin pigmentation is one of the few genuine exceptions: melanin density varies along a cline of solar intensity, from the equator outward, and that variation is real, measurable and consequential.

But a cline is not a race. Fitzpatrick types IV to VI take in South Asia, the Gulf, North and East Africa, Aboriginal Australia, Indigenous America and much of Latin America. "Black", as a social category, spans Fitzpatrick III

to VI. The two do not map onto one another, and this series does not pretend they do.



Where a physiological claim appears in these pages it refers to melanin and to what melanin does its optical behaviour, its response to visible light, the activity of the melanocytes that produce it, and the pigment they deposit after injury. Those claims are defensible and the clinical literature supports them. Claims that Black skin is innately more fragile, drier, or less able to maintain itself are not defensible, and where the beauty trade repeats them, this piece does not.

That distinction is not a technicality. Western medicine has a long and specific history of attributing to Black bodies an innate sickness that turned out, on inspection, to be an artefact of the conditions those

bodies were kept in and the cosmetic industry that grew out of that medicine inherited the habit along with the science.

> ***The charge in this series is not that the beauty industry overlooked a racial difference. It is that it never tested against the range of skin it sells to, and then reached for race selectively, whenever a marketing story required it.***

The Science Gap: When Research Ignores Reality

If the funding landscape reveals economic exclusion, the clinical research landscape reveals something more insidious: a systematic failure to understand how skincare actually works on darker skin tones. The implications extend from irritating white casts on sunscreen to life-threatening misdiagnoses of skin conditions.

A review of 246 dermatology clinical trials registered on ClinicalTrials.gov between 2017 and 2021 found that whilst Black Americans comprise 13.6% of the US population, they accounted for just 7.9% of trial participants. Only 21.9% of the studies analysed reached or exceeded that 13.6% threshold.[12]

More recent research confirms these disparities persist, with a February 2024 Journal of the American Academy of Dermatology issue highlighting the continuing scarcity of clinical research in skin of colour.[13]

The educational deficit compounds clinical disparities, as medical students report low confidence in diagnosing and treating conditions in melanin-rich skin due to inadequate curriculum coverage.[14] And the shortfall is sharpest precisely where it should be smallest. Measured against the conditions Black patients disproportionately carry, Black

participation ran to 12.6% of acne trials, 11.6% in atopic dermatitis, 7.9% in alopecia, 2.9% in psoriasis and 0.6% in rosacea. The research thins out fastest in the diseases most likely to walk through the door.[15]

What Doctors Were Taught

Low confidence is the benign reading. When researchers measured what medical students actually believe, they found something worse than uncertainty.

Surveying students and residents at the University of Virginia in 2016, Kelly Hoffman and colleagues found that 37.3% of third-years endorsed the proposition that Black skin is thicker than white skin; 38.9% of second-years believed Black people age more slowly; 19.4% believed Black nerve endings are less sensitive.[50]

These are not gaps in knowledge. They are inherited claims the last of them traceable directly to the antebellum medical literature on Black insensitivity to pain still carried into practice by roughly a third of the doctors formed at one of America's better medical schools. The evolutionary biologist Joseph Graves, reviewing the same study, adds a further turn: several of the propositions its author's classified as true were themselves false.

> **The audit needed auditing.**

The Missing Faces

The educational pipeline compounds these problems. A 2018 analysis of 4,146 images across four preclinical anatomy textbooks used at top medical schools found that only 4.5% depicted dark skin tones.[17] Six years later a separate team repeated the exercise across eleven

anatomy atlases and textbooks published between 2015 and 2020 a larger sample, newer editions, different researchers.

They arrived at the same figure: 4.5%.[18]

Dermatology residents report feeling unprepared to treat skin of colour upon completing their training. When doctors cannot recognise how conditions present on darker skin, when their textbooks show them only lighter-skinned examples, misdiagnosis becomes inevitable.

This matters beyond aesthetics. Patients with skin of colour present with more advanced skin cancer at diagnosis 16% against 5% and their five-year survival rate is 66.2%, compared with 90.1% for white patients.[19] The myth that darker skin doesn't require sun protection has contributed to this disparity — a myth reinforced by an industry that, until recently, produced sunscreens that left visible white residue on darker skin tones, effectively making the product unusable for the consumers who needed education about its importance.

NEXT IN THE SKINCARE INVESTIGATION

03 What the Ingredient List Reveals

The research gap is one part of the story.

But what happens when an industry that hasn't adequately studied the full range of skin it sells to turns that incomplete understanding into products?

In Part 3, the investigation moves from the clinical trial and the medical textbook to something sitting in almost every bathroom:

the ingredient list.

And the story begins with an experiment conducted in 1988.

Source notes

12. Review of 246 dermatology clinical trials registered on ClinicalTrials.gov, 2017–2021 (as cited in the master research).

13. Journal of the American Academy of Dermatology, February 2024 issue on clinical research in skin of colour (as cited in the master research).

14. Research on medical students' confidence diagnosing and treating melanin-rich skin (as cited in the master research).

15. Disease-specific participation data for acne, atopic dermatitis, alopecia, psoriasis and rosacea trials (as cited in the master research).

17. 2018 analysis of 4,146 images in four preclinical anatomy textbooks (as cited in the master research).

18. Later analysis of eleven anatomy atlases/textbooks published 2015–2020 (as cited in the master research).

19. Skin-cancer stage-at-diagnosis and five-year survival comparisons (as cited in the master research).

50. Hoffman et al. (2016), study of false beliefs about biological differences among medical students and residents; contextual discussion by Joseph Graves (as cited in the master research).

Part 3 bridge: Berardesca/Maibach 1988 experiment involving sodium lauryl sulfate and Black and white volunteers (as cited in the master research).

ABOUT THIS RESEARCH

Diversity in Crisis: *The Skincare Investigation*

A six-part investigation into how the beauty industry's promises on inclusion compare with what it actually does, written by Boyowa Olugbo, Managing Director of Praxis Brands.

01	The $9.4 Billion Customer Beauty Keeps Undervaluing	PUBLISHED 11 AUGUST 2026
02	**The Skin Skincare Forgot to Study**	**PUBLISHED 30 AUGUST 2026**
03	What the Ingredient List Reveals	PUBLISHED 4 SEPTEMBER 2026
04	The Luxury Hotel Bathroom Has a Problem	PUBLISHED 24 SEPTEMBER 2026
05	When Perfumers Play Dermatologist	PUBLISHED 20 OCTOBER 2026
06	The Shelf Space Struggle	FORTHCOMING

How it is researched

Factual claims carry numbered notes. Ingredient lists are taken from manufacturers' own published information, with the date accessed recorded, and cross-checked against at least two independent sources. A claim that rests on a single source is removed rather than qualified.

Our interests, stated

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If you think we are wrong

Write to boyowa@praxisbrands.co.uk with what you think is wrong and, where you can, the evidence. We will check it, correct the research if we got it wrong, and record the correction at praxisbrands.co.uk/research.

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